

Three new patients and ten minutes to plan

Who to see first, what to do first, and why the calmest vital signs on the board can belong to a patient at real risk.

CASE IN BRIEF

On an ED day shift you were assigned three new patients and asked for a plan from the triage charts alone, before any history. Jordan Liu, 18, has known asthma, an SpO2 of 89%, and audible wheeze. Alex Rivera, 32, has two days of worsening leg weakness and normal vital signs. Camden Pierce, 45, has a hot, swollen right knee with fever, tachycardia, and a positive sepsis screen. Jordan is failing to oxygenate now. The central problem is recognizing that Alex, whose numbers look the best, comes next: Guillain-Barré syndrome can progress to respiratory failure before the oxygen saturation falls.

Who to see first

PATIENT	ORDER	WHAT SET THE URGENCY
Jordan Liu	First	A breathing problem now: hypoxemia, tachypnea, tachycardia, and wheeze despite home rescue inhaler use. He needs treatment and a measured response immediately.
Alex Rivera	Second	A breathing problem that may be coming. Weakness that worsens by the day and now limits walking can reach the breathing muscles, and respiratory failure can occur without breathlessness. Normal vital signs do not lower the priority.
Camden Pierce	Third	Circulation is holding: fever and tachycardia with a normal blood pressure. Septic arthritis until proven otherwise, and the joint fluid and the antibiotics that follow are time-sensitive.

Placing Alex last because the vital signs are normal is the error this case is built to catch.

A model you can apply to any board

The ABCDE approach assesses Airway, Breathing, Circulation, Disability, and Exposure in that order, treating each life-threatening problem as it is found and reassessing after every intervention or change.¹ Applied to triage charts, it sorts patients by which system is failing first. Add one question it does not ask on its own: what could fail next?

APPLIED TO THIS BOARD

Breathing, failing now

Jordan. SpO2 89%, RR 24, wheeze. Treat bronchospasm first.

Breathing, threatened

Alex. Breathing is normal on the chart, but the weakness (a disability finding) points back to breathing. Measure the breathing muscles rather than wait for the saturation to fall.

Circulation, holding

Camden. Fever and tachycardia with a normal blood pressure. Infection that needs a source and treatment soon, not a failing circulation.

Triage acuity

Triage nurses assign Emergency Severity Index levels. Patients at ESI levels 1 and 2 are the most often admitted to the ICU.² Acuity is about risk, not only about the numbers on the chart.

What to do first, and what can wait

JORDAN: TREAT, THEN MEASURE THE RESPONSE

Oxygen, a short-acting beta-agonist every 20 minutes for the first hour, and a systemic corticosteroid

The core of initial treatment. The steroid is part of the first hour, not a decision for later.³

Peak flow or FEV1

An objective measure tells you whether treatment is working and later whether he can go home: improved symptoms and 60 to 80% of predicted.³

Before any discharge

Inhaled corticosteroid or a step up in maintenance therapy, an action plan, and close follow-up.³

ALEX: MEASURE THE BREATHING MUSCLES

Vital capacity, maximal inspiratory pressure, single-breath count, cough and swallow

Bedside measures show respiratory muscle weakness before the saturation does. Pair them with cardiac monitoring, because autonomic instability can cause arrhythmias and blood pressure swings.⁴

Basic labs and an early lumbar puncture

Mainly to exclude other causes of acute weakness. A normal CSF protein early in the illness does not rule the diagnosis out.⁴

Can wait, or should not be given

MRI is not routine and serves to exclude mimics, so it does not come before the breathing check. Corticosteroids are not recommended.^{4,5}

CAMDEN: FLUID BEFORE ANTIBIOTICS

Arthrocentesis with cell count, Gram stain, culture, and crystals

Synovial fluid is required to confirm the diagnosis. ESR and CRP help but are nonspecific.⁶

Empiric antibiotics once the fluid is obtained

The order matters. In a stable patient, draw the fluid first, then start antibiotics without delay.⁶

The diagnosis not to miss

JORDAN

A severe exacerbation

If he is not better after the first hour, he is moving toward severe disease, where adding ipratropium and magnesium is associated with fewer admissions.³

ALEX

Neuromuscular respiratory failure

Concern rises with vital capacity below 20 mL/kg, maximal inspiratory pressure weaker than 30 cmH₂O, a single-breath count of 19 or less, rising respiratory or heart rate, or a weak cough or swallow.⁴ Any of these goes to your senior now and points toward the ICU.

CAMDEN

Septic arthritis

A hot, swollen joint with fever is joint infection until the fluid says otherwise. Delay in diagnosis and treatment can leave permanent damage.⁶

Three take-home points

CARRY IT FORWARD

With your next full board, treat what is failing now, then ask what could fail next before you decide who can wait.

- 1 Vital signs describe the present; the history describes the trajectory.** Sort by what is failing now, then by what is threatened. A patient whose function is worsening can outrank one with abnormal but stable numbers.
- 2 Deciding who goes first and doing each task well are separate skills.** Give a reason for the patient you see first, and also say why the others can safely wait. That second sentence is where weak prioritization shows.
- 3 Sequence is part of the plan.** Some actions must come before others: fluid before antibiotics, treatment before reassessment, the breathing check before the scan. Write your first actions in the order you would do them.

References

TRIAGE AND PRIORITIZATION

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CLINICAL GUIDANCE

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FURTHER READING

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